Utilization Through Low-Cost / High Impact Community Pilot

Introduction

PHIERS.org is committed to improving the impact and reach of the state-run Health Information Exchange (HIE), by picking up where hospitals and primary care providers leave off – in the community. We can accomplish these goals by making lab, digital images and pharmacy data accessible to low cost patient-centered medical homes and expanded care teams for high risk patients, in a secure, scalable and cost-effective manner. Click HERE to learn about our veterans-focused pilot.

Working together with subject matter experts in discharge planning, case management and home health services, we have developed an innovative pilot that will provide chronically ill patients with the resources they need to receive more comprehensive and better coordinated care across settings and between multi-disciplinary care teams. We will use a HIPAA compliant Personal Health Record (PHR) to improve safety, quality and efficiency, while tracking quality measures to compute the cost-benefits of our program, over time.

The target audience is comprised of vulnerable, high need patients (on Medicaid and/or Medicare), with multiple chronic conditions. This population is the most likely to receive fragmented and uncoordinated care. They are the most likely to be readmitted and represent the fastest growing / costliest group of health care consumers.

3-phase Program

Phase 1 will focus on veterans who receive primary care from private doctors and mental health providers. **Phase 2** will expand and serve the needs of dual eligible seniors, with a focus on patients living with multiple chronic conditions. **Phase 3** will serve the needs of Medicare and Medicaid patients across the state. All phases will include a membership drive to attract primary care providers, mental health professionals and community-based nonprofit organizations. Participants will be eligible for performance-based grants for participating in our program. Expected outcomes include a dramatic increase in the number of subscriptions to the CRISP HIE as well as a sharp increase in the number of queries. An option also exists to improve the quality of data stored by the HIE, by leveraging the longitudinal health information in the PHR when patient's visit the ED.

Statistics

- **MedCHI currently serves ~25,000 Medicare beneficiaries**, which represents about 3.2 percent of the total number of Medicare beneficiaries in Maryland. We believe we can help them attract large numbers of Medicare and Medicaid beneficiaries.
  - Maryland Medicare covers 778,844 Maryland residents
  - Medicaid covers 847,900 Maryland residents and
  - 432,000 receive benefits from both Medicare and Medicaid (**dual-eligibles**).
- **Regarding Maryland's Veterans and Veterans Choice…**
  - There are more than 55,000 veterans served by Maryland's VA, and many are using private doctors to speed up access, and receiving uncoordinated services.
  - Mental Health statistics: About 95,200 (20%) of veterans of the Iraq and Afghanistan wars have been diagnosed with PTSD. About 143,000 (30%) of soldiers developmental problems within 3 to 4 months of being home.
- **Regarding the Evergreen Coop,** they serve 15,000 patients, and aims to compete with health care giants. **We can help.**

The Public Health Information Exchange and Referral Service (PHIERS) combines a HIPAA-compliant Personal Health Record with resource matching and referral services that improve the quality, continuity and coordination of care offered to patients by helping providers and nonprofits form a low cost and patient-centered medical home and a comprehensive care team.

What we do…

- **Help patients, caregivers and case managers form useful partnerships** with local resources that provide supportive services needed to improve the patient experience, fill unmet needs, increasing the likelihood of adherence to care plans.
- **Improve the quality of care transitions** by combining health education, decision aides, triage support and mobile health technology in ways that improve the quality of home health care.
- **Reduce the cost of care** by helping patients better adhere to their care plans, reducing complications, ER visits and Readmissions .

About Us…

Through the use of a HIPAA-compliant and non-tethered PHR, we will equip, train and reward patients for presenting their PHR to providers prior to each visit. When combined with a shared case management service, resource matching and referral tools, case managers will be equipped to better coordinate care and track outcomes across the continuum.

- Enables members of the patient-centered medical/health home to easily share observations and case notes, reinforce desired behaviors and use outcomes to compute cost-benefit on a patient, organization or program basis.
- Allows primary care providers, which typically operate in isolation from one another, to leverage the strengths of the community, and in so doing gain the benefits of a low cost patient-centered medical (and health) home.
- This program is designed to help CRISP, Evergreen and participating physicians to function like a state-wide Accountable Care Organization (ACO), but for a fraction of projected cost. This will improve the safety, quality, efficiency, cost-effectiveness, hence competitiveness, of Maryland's public health and social safety net.

The PHIERSafe PHR can use DIRECT messaging to share data with physician EMRs and hospital EHRs. When a patient presents at an ED, this data can then be securely uploaded into the hospital's EHR, as well as CRISP. This will improve the quality of data stored in and shared by the CRISP HIE with subscribing physicians in the community, resulting in a more complete picture of the patient and their health. The result in a less fragmentation and easier care coordination, resulting in better adherence and outcomes, fewer readmissions and cost savings directly attributed to our program.

In this way, we believe that the quality of the data shared by CRISP will be better than its competitors, which will enable Maryland's public health infrastructure to out-perform other states in their quest to adhere to CMS goals for value-based payment options. It also delivers on the ONC's goals for rapid adoption of Health/IT among physicians and patients. Working together, we believe CRISP will lead the industry, and Maryland will attract national recognition and grants.

Our Team

- **PHIERS.org** is a nonprofit coop that provides tools, training, supportive services and promotes collaboration between patients; public health and social safety net organizations while building healthier communities and applying for grants.
- **Daedalus Systems,** a veteran-owned, veteran-physician led consultancy that provides subject matter expertise in the areas of emergency care, discharge planning and transitional care.
- **Subject Matter Experts from VA Medical Centers and leading safety net health and hospital systems** provided guidance in the areas of clinical care coordination, home health services and caregiver support.

Beneficiaries

- **Patients and Caregivers:** Decision aides, secure health/IT and tools to promote collaboration will help provide the benefits of an expanded care team, while connecting patients to the supportive services they need to improve outcomes and the patient's experience. This is of particular important to veterans and poor seniors with complex chronic conditions.
- **Case Managers and Home Health Agencies:** Tools to improve the quality of assessments and referrals, view patient records, develop care plans, schedule transportation, prepare for medical appointments, etc. are provided by the PHIERPlace decision support system.
- **Outpatient Clinics and Local Hospitals:** Patients will have improved access to the resources and supportive services needed to increase adherence to care plans, reducing the likelihood of complications, ER visits and readmissions.

Conclusion

Building upon the VA's PHIER-powered pilot, public HIT's have an opportunity to lead the industry by improving the quality of data exchanged between hospitals and PCPs without disruptive change. The entire public health and social network will benefit from the use of tools that leverage the strengths of the local community and collaboration across multi-disciplinary care teams. This is an effective way to reduce healthcare costs without harming care. We are ready!

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